

Ethics Behind the Treatment of Mentally Ill Inmates in the U.S. State and Federal Prisons

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Introduction

The United States of America has the highest incarceration rate in the world despite making up less than 5% of the world's population (Wagner & Bertram).¹ In 2020, the inmate population inside its prison system was approximately 2.2 million with “54 percent of state prisoners and 45 percent of federal prisoners reporting mental health concerns” (Collier 56).² Some of these concerns include mania, major depression, and psychotic disorders (James & Glaze)³. The cause for these high numbers of unmet psychological needs can be attributed to the lack of a sufficient healthcare system for emotional health and an unequal provision of services for inmates suffering with mental illness. It is not unreasonable to conclude that U.S. prisons have essentially become mental health institutions, a belief shared by scholars such as Daniel from the American Academy of Psychiatry and Gostin et al from the Law and National Library of Medicine. In spite of this, only “[six] in 10 mentally ill prisoners received treatment while incarcerated in a state or federal prison,” of which were unevenly distributed based on characteristics such as sex and race (Gostin et al). Additionally, inmates with mental illness face harsher treatment compared to those that do not. With such concerning circumstances in regards to the wellbeing of inmates, the ethical standards for aiding mentally ill prisoners inside US state and federal prisons must be called into question and reassessed.

¹ “Peter Wagner is an attorney and the Executive Director of the Prison Policy Initiative” and Wanda Bertram is an “expert on tablet computers in prisons, and has written [...] about prison tablet contracts, bans on physical mail behind bars, and other policies that force incarcerated people and their families to shoulder the costs of incarceration” (Prison Policy Initiative).

² Lorna Collier is an experienced writer and researcher for the American Psychological Association, “which is the leading scientific and professional organization representing psychology in the United States, with more than 133,000 researchers, educators, clinicians, consultants, and students as its members” (American Psychological Association).

³ Doris J. James and Lauren E. Glaze are both statisticians for the Bureau of Justice Statistics. They wrote the 2006 special report “Mental Health Problems of Prison and Jail Inmates”, which was peer reviewed by many other professionals working for the BJS.

Lack of healthcare to aid mentally ill inmates

The healthcare resources and services provided in US prisons are not sufficient enough to help inmates suffering with mental illness, as medication, counseling, and other forms of treatment are not appropriately given to them. According to a research study conducted by multiple scholars⁴ with credentials in medicine and healthcare, “[among] inmates with a previously diagnosed mental condition who had been treated with a psychiatric medication in the past, 69.1% [...] of federal [and] 68.6% [...] of state [...] inmates had taken a medication for a mental condition since incarceration” (Wilper et al 666-672). These statistics may appear acceptable, but it does not apply for the many prisoners that are mentally ill without a diagnosis. Furthermore, there is no certainty that after an inmate has taken medication once, treatment will continuously be given to them. This deficiency in welfare has been an ongoing issue, as when juxtaposing data from the Bureau of Justice Statistics – an official government program under the U.S. Department of Justice – about the treatment of mentally ill inmates from 1999 and 2017, there were barely any improvements despite the 18 year difference. In the report from 1999, “[fifty] percent [of federal and state inmates] said they had taken prescription medication; 44% had received counseling or therapy” (Ditton).⁵ Comparably, the 2017 report states that “[an] estimated 46% of prisoners [with serious psychological distress...] had received prescription medication, 42% had received counseling or therapy, and 34% had received prescription

⁴ “Andrew P. Wilper, Steffie Woolhandler, Karen E. Lasser, Danny McCormick, David H. Bor, and David U. Himmelstein were with the Department of Medicine, Cambridge Health Alliance, Cambridge, MA. J. [at the time of this study and] Wesley Boyd is with the Department of Psychiatry, Cambridge Health Alliance, Cambridge” (American Journal of Public Health).

⁵ Paula M. Ditton is a statistician for the Bureau of Justice Statistics. She wrote the 1999 special report “Mental Health and Treatment of Inmates and Probationers”, which was peer reviewed by many other professionals working for the BJS.

medication and counseling or therapy” (Bronson & Berzofsky).⁶ From this, it appears that less inmates are getting the assistance they need in recent times than they did nearly two decades ago.

In cases where mentally ill inmates are provided treatment, it is rarely of good quality. A report conducted by mental health professionals⁷ on behalf of the National Institute of Justice asserts that

“only 15 percent of individuals with [serious mental illness] receive care that could be considered at least ‘minimally adequate’” (Shaffer et al). Evidence of such quality can be found in the high rates of recidivism, roughly 50 percent, only a few years after release due to the ineffective aid the prison provided the inmates while they were incarcerated (Daniel 406-410). Samantha Hoke, a certified psychiatric-mental health nurse, working for the U.S. Department of Justice Federal Bureau of Prisons, suggests possible reasons why the prison’s mental health care is inadequate, including a “lack of resources, [...] conflicts of interest between healthcare providers and security protocols” (Hoke). Although it is unclear exactly what the quality of care is, it can be reasonably inferred that it is substandard in most cases.

Even with these issues, it is necessary to acknowledge that the prisons themselves are not entirely in control of the quality of their health services, as a lack of ample funding poses a challenge in improving it. Brett Mattson, the legislative director for justice and public safety for the National Association of Counties, reports that “Congress has authorized [the Justice and

⁶ Jennifer Bronson is a statistician for the Bureau of Justice Statistics and Marcus Berzofsky is a senior research statistician working for the Research Triangle Institute. They wrote the 2017 special report “Indicators of Mental Health Problems Reported by Prisoners and Jail Inmates, 2011-12”, which was peer reviewed by many other professionals working for the BJS.

⁷“John S. Shaffer [...] served 31 years with the Pennsylvania Department of Corrections [...] and has] expertise [in] technology evaluation, correctional health care services, inmate telephone systems and e-services, and organizational audits” (Priority Criminal Justice Needs Initiative). “Joe Russo is a researcher with the University of Denver [...] He has served both the New York City Departments of Correction and Probation” (Priority Criminal Justice Needs Initiative). “Dulani Woods is a data science practitioner[...] His primary areas of research have included homeland security and justice policy” (Priority Criminal Justice Needs Initiative). “Brian A. Jackson is a senior physical scientist at the RAND Corporation. His research focuses on criminal justice, homeland security, and terrorism preparedness” (Priority Criminal Justice Needs Initiative).

Mental Health Collaboration Program] \$50 million since its inception, [however], appropriators have never fully funded the grant program [...currently], JMHCP is funded at \$40 million” (Mattson). Although this is a statistic for jails, since the amount of funding prisons receive for psychological healthcare is imprecise and ambiguous, this is one of the best frames of reference that can be used to estimate prison funding.

Differing treatment of inmates with mental illnesses

The distribution of assistance for mentally ill prisoners is unevenly given out based on what demographics they fall under. For instance, white state prison inmates generally receive more psychological healthcare compared to black or Hispanic convicts despite the latter groups representing more of the prison population (Ditton). Furthermore, female and male inmates experience different amounts of treatments. The 2006 Bureau of Justice Statistic Report’s data supports the findings of a study conducted by scholars⁸ who have qualifications in healthcare and criminal justice, that is “ 20.1% of men and 24.8% of women in Michigan prisons have a substantial level of [Mental Health] symptoms and that 16.5% and 28.9%, respectively, are receiving MH services” (Fries et al 316-325). However, it is important to acknowledge that female and white inmates are much more likely to report psychological health concerns than their counterparts, potentially contributing to why they receive greater amounts of care (James & Glaze). According to David Lovell, a doctor for the department of psychosocial and community health at the University of Washington, along with other professionals of psychiatry, “women

⁸ Brant E.Fries is a member of the Institute of Gerontology and School of Public Health at the University of Michigan. Angela Schmorrow is also a part of the Institute of Gerontology at the University of Michigan. Sylvia W.Lang works for the Arbor Research Collaborative for Health. Philip M.Margolis is a scholar in the Department of Psychiatry at the University of Michigan. Julia Heany works for the Center For Healthcare Excellence at the Michigan Public Health Institute. Greg P.Brown is part of the Department of Criminal Justice at Nipissing University. Howard E.Barbaree is a member of the Waypoint Centre for Mental Health Care. John P.Hirdesh is a scholar in the Department of Health Studies and Gerontology at the University of Waterloo.

inmates may seek and obtain prison mental health services because of traumatic or stressful events more than men do” (Lovell et al 1290-1296).

Mentally ill inmates also face harsher evaluations compared to inmates that are not. They receive longer prison sentences of an average of “12 months longer than other offenders” due to mental impairments increasing their chances of poor behavior (Ditton). These convicts also have less likelihood of receiving parole, demonstrated by a study published by experts⁹ of law and criminal justice, as there are many indirect mental health factors – such as impulsivity – contributing to inmates scoring “poorly on criminal justice risk assessments [...so if] the parole board is basing decisions primarily on criminal risk, then mentally ill inmates would be less likely to be paroled” (Houser et al). The ethics behind this indirect discrimination are uncertain, as although mentally ill inmates suffer from it, the parole board uses an “objective [...] evidence-based risk assessment in their decision making”, thus it would be inaccurate to suggest inmates are not given parole specifically because they are mentally ill (Houser et al).

Conclusion

Currently, the U.S. state and federal prisons are not equipped to properly aid inmates suffering with mental illnesses. That must be remedied in order to prevent harmful behavior from prisoners whilst they are incarcerated and reoffending after they finish their sentence. A possible solution is to increase funding for health services such as Medicaid. Most indigent prisoners have their healthcare plans canceled once they get incarcerated, and are barely able to pay for the costs of medical services with their low wages (Collier 56). Scholars of the American Journal of Public Health agree with this proposal, stating that “maintaining Medicaid eligibility for

⁹ Kimberly A. Houser and Christine A. Saum both work in the Department of Law and Justice Studies in Rowan University. E. Rely Vilcică and Matthew L. Hiller both work in the Department of Criminal Justice at Temple University, Philadelphia.

inmates” is an effective method of improvement (Wilper et al 666-672). The ethical standards in U.S. prisons must be reconsidered and incorporate active procedures that focus on increasing access and quality of mental health services. In doing so, recidivism rates will decrease and society in the U.S. can be enhanced.

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